

1. Administrative & Study Identification

Full Study Title: Imfinzi BTC Japan PMS_Japan Post-Marketing Surveillance (PMS) Study - CEI/SCEI **Short Title/Acronym:** Imfinzi BTC Japan PMS **Protocol Number:** D933AC00005 **NCT Number:** NCT05835778 **Sponsor Name:** AstraZeneca **Investigational Product:** IMFINZI (durvalumab) Intravenous Infusion (120mg, 500mg) in combination with gemcitabine hydrochloride and cisplatin **Phase of Development:** Post-Marketing Surveillance (Phase 4 / Observational) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To investigate the onset of adverse drug reactions in patients with curatively unresectable biliary tract cancer who receive IMFINZI in combination with gemcitabine hydrochloride and cisplatin under actual use in the post-marketing setting. **Secondary Objectives:** To evaluate specific safety specifications defined in the Japan Risk Management Plan of IMFINZI, including the incidence of Interstitial Lung Disease (ILD), severe diarrhea/colitis, hepatic function disorders, and endocrine disorders.

2.2 Background & Rationale This investigation is conducted as one of the additional pharmacovigilance activities under the Japan Risk Management Plan of IMFINZI. It is performed in compliance with the Ministerial Ordinance on Good Post-marketing Study Practice (GPSP Ordinance) for the purpose of application for reexamination under Article 14-4 of the Act on Securing Quality, Efficacy, and Safety of Products Including Pharmaceuticals and Medical Devices in Japan.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Observational (Post-Marketing Surveillance) **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration **Target \$N\$:** 236 patients **Number of Sites:** Multiple sites in Japan (including Hokkaido) **Estimated Study Start:** 06-Jun-2023 **Estimated Primary Completion:** 21-Nov-2025

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Patients with curatively unresectable biliary tract cancer. 2. Patients receiving IMFINZI for the first time in combination with gemcitabine hydrochloride and cisplatin.

4.2 Exclusion Criteria 1. No explicit exclusion criteria were specified in the protocol (all real-world indicated patients meeting inclusion criteria are eligible).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: IMFINZI Intravenous Infusion 120mg or 500mg administered according to the approved real-world dosing schedule. **Route of Administration:** Intravenous (IV) **Duration of Treatment:** Until disease progression, unacceptable toxicity, or decision to discontinue per standard clinical practice.

5.2 Concomitant & Prohibited Therapy Permitted: Gemcitabine hydrochloride and cisplatin as part of the combination therapy. Concomitant medications for symptom management allowed per standard of care. **Prohibited:** Any therapies strictly conflicting with the real-world prescribing information for IMFINZI.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening / Baseline	Day 0	Patient consent/enrollment, First Dose of IMFINZI + chemotherapy, Baseline Assessments
Interim	Per Clinical Practice	Routine ongoing actual-use monitoring, Safety and Adverse Drug Reaction recording
End of Study	Nov 2025	Final safety documentation, Data collection cutoff for reexamination application

(Note: As an observational PMS study, specific visit windows align strictly with the patients' routine clinical care schedules.)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence and onset of adverse drug reactions under actual use in the post-marketing setting. **Measurement Method:** Pharmacovigilance reporting collected from clinical sites through GPSP-compliant data collection mechanisms.

7.2 Secondary & Exploratory Endpoints Incidence of pre-defined safety specifications, including: Interstitial lung disease, Colitis/Severe diarrhoea, Hepatic function disorder/Hepatitis/Sclerosing Cholangitis, Endocrine disorders (Thyroid, adrenal,

pituitary), Type 1 diabetes mellitus, Renal disorder, Myositis, Myocarditis, Infusion reactions, and others.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard pharmacovigilance collection via site reporting under the Japan Risk Management Plan. **Serious Adverse Events (SAEs):** Handled and expedited per Japanese regulatory requirements (GPSP Ordinance). **Data Monitoring Committee (DMC):** Evaluated internally by the AstraZeneca Pharmacovigilance team and Japanese regulatory authorities.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients who received at least one dose of the investigational product in the real-world setting). **Sample Size Justification:** A target of 236 patients to provide adequate real-world exposure data for regulatory reexamination. **Handling of Missing Data:** Standard handling for observational registries; event rates calculated based on evaluable follow-up data.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study is conducted in accordance with the Ministerial Ordinance on Good Post-marketing Study Practice (GPSP Ordinance) in Japan. **Monitoring Plan:** Routine review of post-marketing case report forms and pharmacovigilance databases. **Audit Trail:** Data collected and compiled securely for application for reexamination under Article 14-4 of the Act on Securing Quality, Efficacy, and Safety of Products.

11. Study Identifiers & Governance

Primary ID: {{D933AC00005}} **Registry Number:** {{NCT05835778}} **Sponsor/Lead Organization:** {{AstraZeneca}} **Study Title:** {{Imfinzi BTC Japan PMS_Japan Post-Marketing Surveillance (PMS) Study - CEI/SCEI}} **Official Regulatory Title:** {{Specific use-results study of IMFINZI Intravenous Infusion 120 mg, 500mg in patients with Curatively unresectable biliary tract cancer}}

12. Clinical Framework (Biliary Tract Cancer Specific)

Primary Condition: {{Curatively unresectable biliary tract cancer}} **Diagnosis Threshold:** {{Confirmed unresectable biliary tract cancer indication}} **Medical Methodology:** {{IMFINZI +

Gemcitabine + Cisplatin Combination Therapy}} **Optimization Target:** {{Adverse drug reaction identification and actual-use safety profiling}}

13. Study Procedures & Methodology

Management Pathway: {{Post-Marketing Surveillance (PMS) under actual clinical use}} **Education Protocol (HCP):** {{Japan Risk Management Plan compliance and GPSP Ordinance adherence}} **Education Protocol (Patient):** {{Routine clinical care instructions for IMFINZI treatment}} **Quality Audit Mechanism:** {{Pharmacovigilance monitoring and adverse event tracking for reexamination application}}

14. Observation & Follow-Up Schedule

Total Duration: {{Follow-up from Jun 2023 to Nov 2025}} **Onsite Visit Cadence:** {{Per routine clinical practice guidelines for BTC patients}} **Remote Monitoring Frequency:** {{Continuous pharmacovigilance reporting}} **Checklist Review Interval:** {{Periodic PMS data collection intervals established by the Sponsor}}

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				